ANX-06 | Therapist guide only

Maintenance and choosing suitable next steps

ANX-06 | A flexible therapist framework, guided by assessment and agreements.

Purpose

Review change relative to goals, identify helpful tools and conditions, and plan continuation and support. Use during treatment or before a professionally assessed ending. Six units do not mean six sessions or determine when treatment ends.

Before use

Return to ANX-01, goals and documented functioning examples. If review already occurred, use SES-S03 or Core 8 instead of duplicate completion. Missing baseline data remain missing, not reconstructed as fact.

A possible working sequence

Open with SES-S01 as needed and close with SES-S02, feedback and a clear plan.

1. Explore changes in the client's life and what remains difficult or unsuitable.

2. Identify tried tools and conditions, including support and resources enabling functioning.

3. Choose personal signs needing attention and challenging situations without treating every worry as recurrence.

4. Plan a possible response, access to help and what to do if the tool is insufficient.

5. Agree on review timing or conditions, and choose continuation, revisiting, a focus change or professionally considered ending.

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Examples for exploration, not ready-made tasks

Examples are fictional. A real client's circumstances, needs and wishes take priority.

Group	A conversation example
Children	A child joins games more but still struggles with new activities. Keep what helped and choose a route back to help with the appropriate adult.
Adolescents	A teenager started several tasks using a tool, but demands changed. Update the plan and support without treating change as failure of learning.
Adults	An adult notices checking again taking time and limiting activity. Revisit formulation and tried tools, arranging assessment as needed without diagnosing from the sign.
Older adults	A person chooses a brief reminder and one of several tried tools. Preserve their comfortable method and desired support without requiring the entire kit.

A key distinction

These sheets are not full risk assessments or emergency plans. Significant worsening or urgent need requires appropriate professional response, not waiting for review. Help arrangements must reflect actual treatment agreements without promising unoffered availability.

A question for shared reflection

What actually helped this person, what do they still need and how can they return to help if chosen tools are insufficient?

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Individual adaptation and scope

An age label does not replace assessment of development, communication, context and preferences.

Group	A possible approach
Children, approximately ages 6 to 11	Use a concrete situation and one question at a time. The therapist can write spoken answers and check understanding. Hear the child's voice too. Adult roles, sharing and consent depend on the setting and need; do not promise absolute confidentiality.
Adolescents, approximately ages 12 to 17	Choose a personal focus and recording method that respects privacy. Explore setting, demands and relationships. Explain sharing and consent limits, and arrange help without making practice a test or demand for obedience.
Adults	Include roles, culture, language, circumstances and resources as described. Do not assume time, money or support is available. Shared choice does not replace clinical suitability assessment.
Older adults	Use respectful adult language, comfortable pace and spacing. Ask about preferred completion methods and help without assuming reduced ability or family dependence. Life experience and preferences inform the work.

Checking understanding and choice

Ask the client to explain their understanding and choice in their words. Shorten, leave fields blank or answer aloud. If questions are unsuitable or burdensome, adapt the approach rather than require completion.

This unit's specific boundary

Forms provide no recovery score, ending threshold or diagnosis. Participation and completion are not evidence of improvement. Decisions about ending, continuing and monitoring are separate from booklet completion and documented in the appropriate clinical setting.

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Practice, follow-up and professional review

Practice is chosen jointly; the sheet does not automatically authorize action.

Before practice

Choose a few useful ANX-06-P reminders from the plan, not a toolbox the client must use in full. Decide on convenient storage and privacy. The card does not replace a full treatment plan.

Back in the session

Review use and suitability at the agreed time. Without improvement or with worsening, return to assessment and response selection, not just more practice. Continue treatment, revisit a unit or refer as needed. Include feedback on pace, relationship and privacy.

Production and review status

Original draft guide and sheets. Professional review for each age and a rights review are needed before distribution. Completed writing is not clinical approval; the kit has not been validated or tested for effectiveness.

General sources for further reading

NICE CG113 — adults
Professional framework for adult generalized anxiety: assessment, functioning and work by trained therapists. It does not validate these six units or their use with children.
https://www.nice.org.uk/guidance/cg113/chapter/Recommendations

NIMH — child anxiety, 2024
Research background on CBT for children; the study used a different protocol. It does not approve this wording or these age adaptations.
https://www.nimh.nih.gov/news/science-updates/2024/cognitive-behavioral-therapy-alters-brain-activity-in-children-with-anxiety

Consulted: 03 Nov 2026. Background sources only, without permission to copy forms or validation of the kit.

Reviewer's name, role, date and versions reviewed:

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